Similarly, Dr Shashank R. Joshi, Padma Shri awardee, noted endocrinologist and a member of Maharashtra's COVID-19 Task Force, in an opinion piece in *The Indian Express* titled 'Smart-locking India' expressed that, 'As health is both a central and state subject, there is an urgent need to clamp down on all major cities and towns, in all aspects: From travel to mass gatherings, to schools, colleges and workplaces. Only then can we prevent an Italy- or China-like situation in India. It's time to halt COVID-19 by smartly locking the country at home so that we can have a better tomorrow. This needs a political will which we currently have.'[2]

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The prime minister chaired a meeting in the second week of March which was attended by most senior officials of the PMO including the principal secretary, the PM's advisors, Cabinet secretary, officers on special duty, secretaries, joint secretaries and high-ranking officials of various departments. In this meeting, he was briefed in detail on the status of the rising cases, the economic impact of a lockdown, the status of essentials and capacity building efforts along with timelines. Consensus was achieved in the meeting that India, in order to protect its people, would need to go into an immediate and complete lockdown. There would have to be a complete shutdown of movement, nationwide.

It was discussed that we would have to start with a 21-day lockdown, monitor the status of the viral transmission as well as the status of our healthcare infrastructure before deciding on any extensions. A huge concern was of our most vulnerable sections of the society, those who sustain their family on daily wages. It was anticipated that lakhs and crores of people would lose their day jobs and would most likely run out of their meagre savings in a matter of days, if any. The prime minister categorically insisted that minute details of how the government would supply food to these workers and their families be planned. The officials discussed an option of the lockdown to be announced in a phased manner so that their approach could be tweaked as per the evolution of the case trajectory. However, the team agreed that if people were given an advance ultimatum, they would travel back to their villages, crowding in public transport and take the infection to remote parts of the country where the state of health infrastructure was extremely poor. Our rural health infrastructure would